NWP – Patient Services

TRAINING MANUAL

Women's Healthcare Associates

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NWP General Information

NWP **West** Phone #: (503) 297-3660 NWP **East** Phone #: (503) 482-1800

Clinic Directions

NWP – West Office

9701 SW Barnes Rd. Ste., 299 Portland, OR 97225

We are located off Barnes Road right by Providence St. Vincent hospital in Southwest Portland. Our office is in the Peterkort Center

Go to the hospital and turn as if you're going to the Emergency room. Instead of going up to the right towards the emergency room parking lot go LEFT toward the Peterkort Center.

NWP - East Office

5050 NE Hoyt St. Ste., 230 Portland, OR 97213

We are located near Hoyt and Glisan right by Providence Portland Medical Center in Northeast Portland. Our office is in the Providence Professional Plaza.

Go to the parking garage and park in level D (as in Delta). There is a sky walk on level D that takes you directly into the second floor of the plaza.

NWP - Gateway Office (1st and 3rd Wednesday of each month)

10566 SE Washington St, Portland, OR 97216

Patients should use either the NWP West or NWP East phone #s to schedule, not the WHA Gateway number

Types of NWP Providers/Specialties/Models of Care

- Behaviorist
- Genetics
- Moderate Risk
- Pediatric Cardiology
- Pediatric Urology- Separate Clinic
- Perinatology
- Satellite Ultrasound Clincs

MFM (Maternal Fetal Medicine) (Perinatologist) Providers

- Lisa Farkouh [LF]*
- Sophia Lannon [SRL]**
- Ashlie Tronnes [AT]*
- Meredith Williams [MW]*
- Laura Sienas [LES]**
- Michael Smrtka [MPS]*

*= Continuity Patient Office days at NWP - **West**

= Continuity Patient Office days at NWP - **East

Genetic Counselors

- Karen Hansen [KH]
- Jeri Milanovich [JM]
- Allison “Ally” Lebenbaum [AL]

MOD Risk Providers – West

- Thomas Brennan [TXB]
- Marni Carlyle [MC] - MAT Provider
- Angela Kondrat [AK] - MAT Provider
- Dafna Lohr [DL] MAT Provider

MOD Risk Providers – East

- Maryanne Garvie-Loveland [MGL] - MAT Provider
- Margaret Gorman [MG]
- Dafna Lohr [DL] - MAT Provider
- Lauren Regueyra [LR]
- Katherine Weyer [KW]

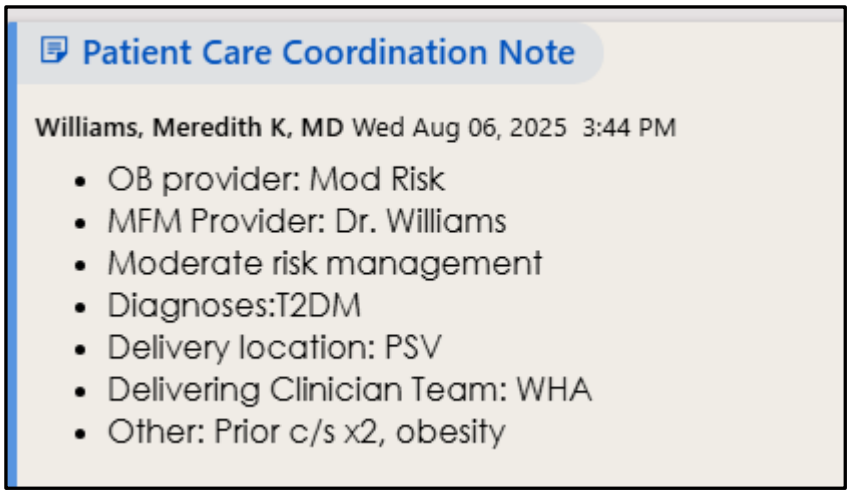
Moderate Risk Providers

Moderate Risk is a group of OB/GYN that work for WHA; they see patients that are at higher risk than the regular OB population. These OB/GYN's have shown an interest in managing higher risk patients, reading Ultrasounds and working closely with the MFM's.

How do you tell if a patient is Moderate Risk?

In Epic there are a couple of ways to tell if a patient is a MOD Risk patient:

1. *Look at the previous appointments of the patient on their appointment desk to see what type of appointment the patient has been scheduled for in the pastfor the current pregnancy.*
2. *Look on the patient's "SnapShot" in their chart Under the Patient Care Coordination Note.*



Example:

This means they are a Perinatal/Mod Risk patient who is seeing one of the Generalist and all scheduling, and phone calls may be handled by NWP. Some patients may be co-managed by their Primary OB/GYN and NWP.

Common Abbreviations

(or see Medical Terms Dictionary in Front Office shared files)

- Advanced maternal age [AMA 35+ yrs]
- After Visit Summary [AVS]
- Bio-Physical Profile [BPP]
- Chorionic Villus Sampling [CVS]
- Fetal Heart Tones [FHT]
- Genetic Consult [GC]
- Gestational Age [GA]
- Last Menstrual Period [LMP]
- Maternal and Fetal Medicine [MFM]
- Moderate Risk Provider [MOD]
- Noninvasive Prenatal Testing [NIPT]
- Non-Stress Test [NST]
- Release Of Information [ROI]
- Sequential [SEQ]
- Telehealth [TH]
- Transvaginal Cervical Length [TVCL]
- Ultrasound [US]
- Q2 = every two (days, weeks, months)

Routing Phone Calls

Needs to speak to a Nurse

- Route ALL non-urgent calls to the **NWPW Advice Nurse** (Voicemail line- ask patient to leave full name, Date of Birth, Phone number to call back and brief message for reason of call)
- **Never** route patients phone calls to individual nurse's phone number
- **Urgent calls** with the following concerns such as bleeding, decreased movement, contractions, blood pressure issues, and leaking fluid please reach out to **NW Perinatal Navigator East or West** to assist in finding an available RN to speak with patient

Patient Returning a call to an NWP Advice Nurse

- Call a PSR first to see if that individual nurse is available for the patients' call. If the nurse is not available, the PSR will give instructions on if you are transferring the call to another available NWP Nurse or if needing to transfer to the NWPW Advice Nurse

Asking to speak to their provider

- All calls will need to be triaged by the Nurses, transfer patient calls to the **NWPW Advice Nurse** (Voicemail line- ask patient to leave full name, Date of Birth, Phone number to call back and brief message for reason of call)
- On a few occasions, our providers do call out to individual patients who return their calls. You can check with the provider if they are expecting that patients call. If not, all calls should be transferred to the NWPW Advice Nurse

Returning a call to a Genetic Counselor

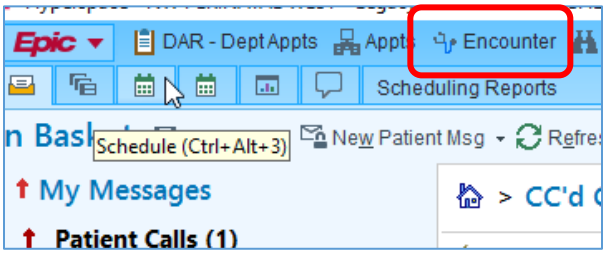
- Patients can reach our Genetic Counselor directly through our automated phone tree options. You can also call the Genetic Counselor directly that the patient wishes to speak with or are returning a call too. If they are unavailable you could try another Genetic Counselor if needed to give out test results

Pharmacy Calls

- Transfer calls to the **NWPW Advice Nurse voicemail line** (Voicemail line- ask patient to leave full name, Date of Birth, Phone number to call back and brief message for reason of call)

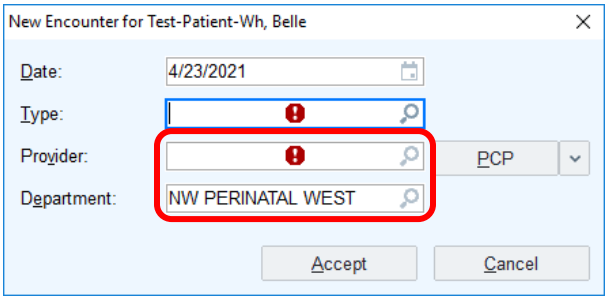
NWP Patient Summary

How to create a NWP Patient Summary



1. Find patient using “Appts” button
2. Click on “Encounter” at the top of the screen to start a new Patient Summary

3. Click on the patient’s name in the patient finder window
4. Select Create an Encounter in the Upper Left corner



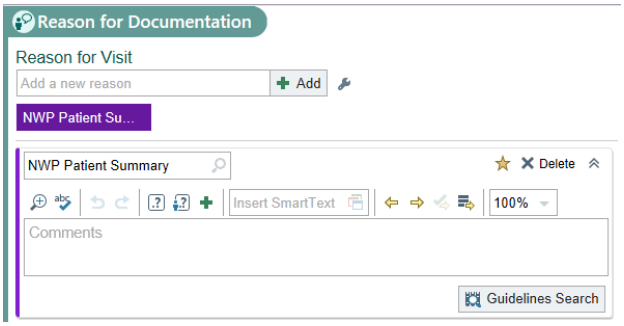
5. Fill out New Encounter information:
   - Type = NWP Patient Summary
   - Provider = Use patients current NWP Provider, if provider unknown use “NONE PER PATIENT”
   - Click “Accept”

6. If a Pregnancy Episode is not created you will need to create one by click on “Episodes”
   - Click on “New Episode”
   - **Name = Pregnancy (if not pregnant use Pre-Pregnancy)**
   - **Type = Pregnancy (if not pregnant use Care Management)**
   - Click “Accept”

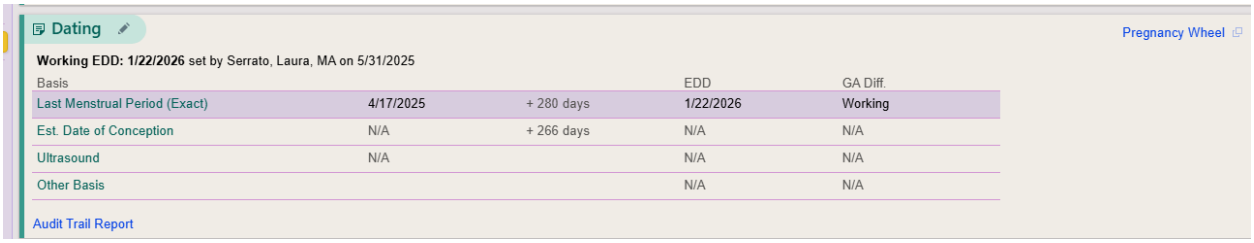




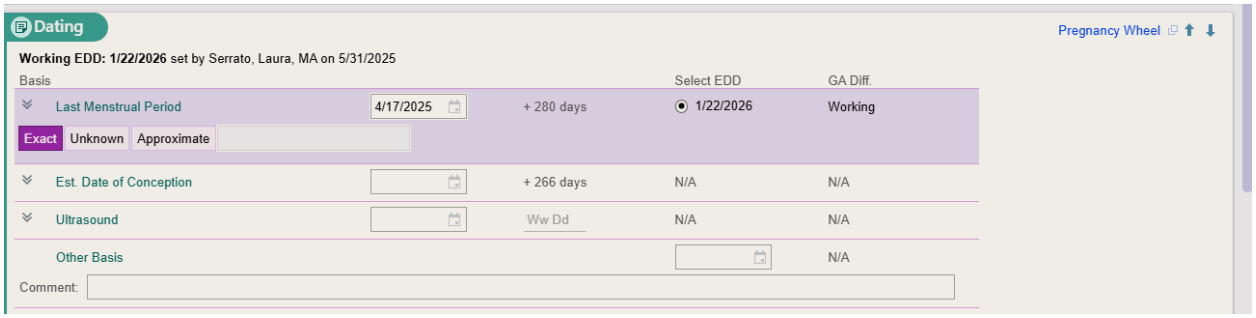
7. Under Reason for Documentation
   - Reason for Visit = **NWP Patient Summary** (comments are not needed)



8. Enter patients dating information under the "Dating" section of the patient summary (found further down the screen) To expand this section, click: Dating in the left hand corner.
9. Enter the dating information you are able to find in the referral, records, or in the US report.



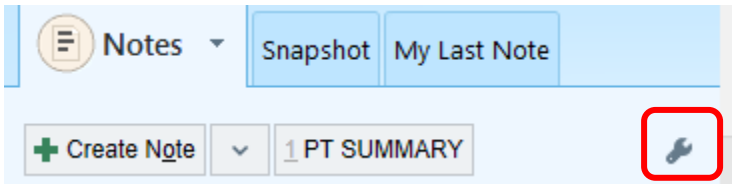
This section has several different options for adding information.



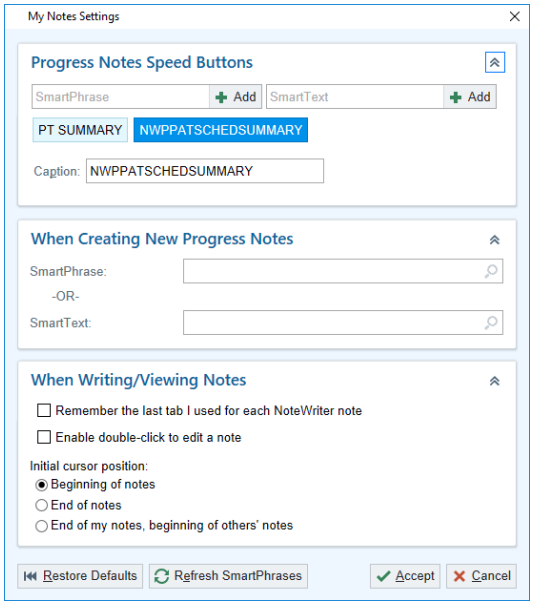
**Please be sure to click on the circle next to the EDD date. If the circle is not selected, the information will not be pulled into the NWP Patient Scheduling Summary SmartPhrase.

Adding the New NWP Patient Scheduling Summary Smart Phrase (quick button)

1. On the far right-hand side of the Notes section of the patient summary, click the wrench

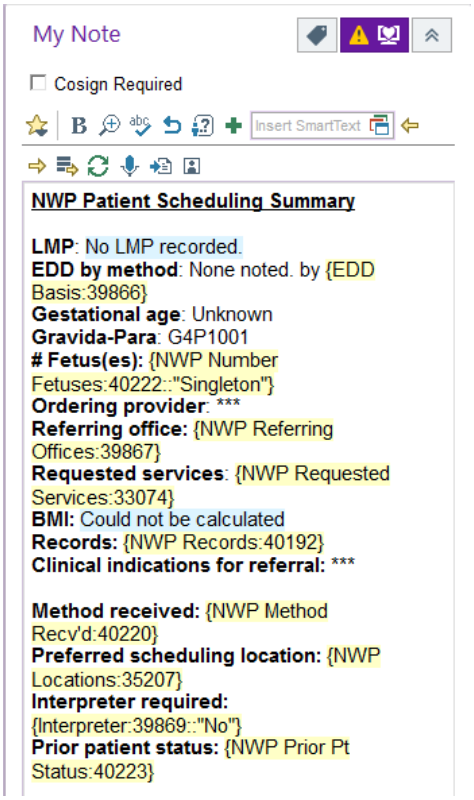


2. Use the SmartPhrase search bar to look for NWP Patient Scheduling Summary (NWPPATSCHEDSUMMARY)
3. Click Accept



NWP Patient Scheduling Summary:

LMP – *(pulls from what is/was already entered in the dating tab or CCC may enter once records reviewed)*



EDD by method: *(if known or CCC may enter once records reviewed)*

- LMP – sure
- LMP – estimated
- Conception date
- Ultrasound
- IVF/Embryo transfer
- ***
- Unavailable

Gestational age: *(pulls from what is/was already entered or CCC may enter once records reviewed)*

Gravida-Para: *(pulls from what is/was already entered or CCC may enter once records reviewed)*

of Fetus (es):

- Singleton
- Twins
- Triplets
- Not currently pregnant
- ***
- Unavailable

Order Provider: Type in ordering/referring provider

Referring Provider:

- Clackamas County Health Clinic

- Multnomah County Health Clinic
- Northwest Primary Care
- Northwest Women's Clinic
- Oregon Reproductive Medicine
- Providence Seaside Clinic
- Providence Women's Clinic – East (East Portland)
- Providence Women's Clinic – East (Milwaukie)
- Providence Women's Clinic – Hood River
- Providence Women's Clinic – West (PSVMC)
- Providence Women's Clinic – West (Progress Ridge)
- Providence Women's Clinic – West (Mercantile)
- The Oregon Clinic
- Virginia Garcia Clinic
- *** (Add NWP West or NWP East if Scheduling Instructions come from NWP)

Requested services: *(select what is being ordered)*

- Ultrasound
- Genetics
- MFM
- Mod Risk
- Blood draw
- ***

BMI: *(pulls from what is/was already entered or CCC may enter once records reviewed)*

Records:

- Care Everywhere
- Hard copy – in provider's Inbox
- Hard copy – under Media tab
- Not received – re-requested

- ***

Clinical Indication for referral: *(Free type what is being order and reason for referral)*

Method Received: *(select how order/referral was received)*

- Fax
- Outside message
- Patient call – pending referral from provider
- Scheduled at patient check out
- ***

Preferred scheduling location:

- NWP – West
- NWP – East
- NWP - Gateway
- No preference
- ***

Interpreter required:

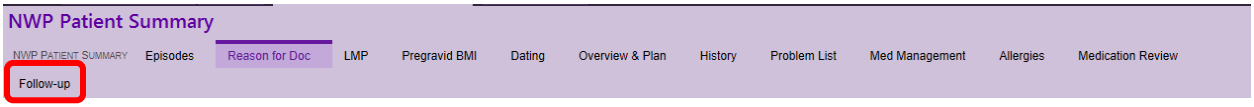
- Interpreter:
  - Yes: {WHA Interpreter Language:}
  - **No**
  - ***
    - WHA Interpreter Language:
      - Spanish
      - Russian
      - Vietnamese
      - Cantonese
      - Arabic
      - ***

Prior patient status:

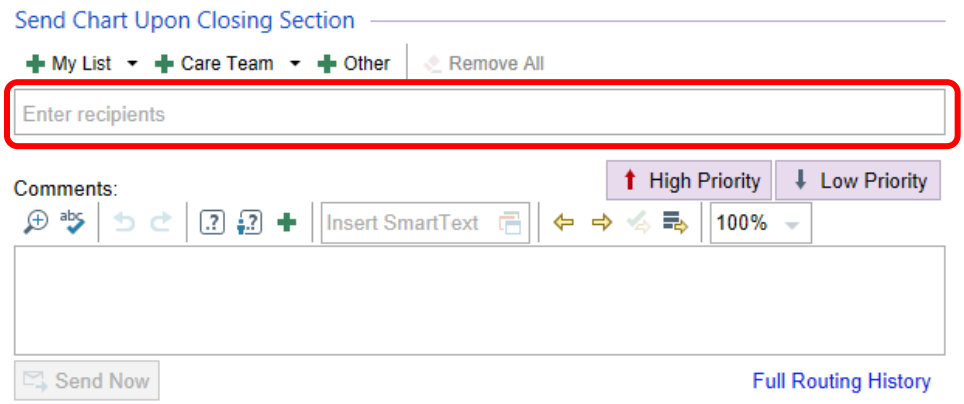
- New patient
- Current patient
- Prior patient
- ***

Routing Patient Summary

1. Click on "Follow-up" in the toolbar on the top of the NWP Patient Summary screen



2. Enter recipients (EX: P WHA Clinical Care Coordinator or P WHA Patient Services NWP Pool)



3. Click "Send Now"

Scheduling Scripting

HIPAA Scripting

- Discussing patient and patient info with any individual who are not listed on the patient's "Permission to Communicate" Form is against our HIPAA Best Practices. If you have any questions about HIPAA or how to troubleshoot a conversation, ask your supervisor

- For urgent medical issues, if a patient is unable to call in themselves because of a medical emergency, it is always ok for nursing staff to TAKE information from whoever is calling in and give directions based on the symptoms they are describing. Specific medical information from the patient's chart should not be shared with unauthorized individuals. If the patient is available in the room, they can give verbal permission to WHA to discuss their specific details with the caller; this should be documented in the phone note.

- Add a Patient Message to patient's appointment desk asking front office to ask patient to review "Permission to Communicate" form at next appointment.

Leaving Voicemail Messages:

**Check to see if patient has "WHA- okay to leave detailed message" which allows us to leave more detailed information.

1st Left message

Hi, this message is for (patient name). My name is (your name) and I'm calling from a doctor's office because we have received a referral to get you scheduled with our clinic. Please give us a call back at 503-297-3660 option 2 for scheduling. Thank you!

2nd Left message – Final attempt

Hi, this message is for (patient name). My name is (your name) and I'm calling from a doctor's office because we have received a referral to get you scheduled with our clinic. Please give us a call back at 503-297-3660 option 2 for scheduling.

I do want to let you know that this is our final attempt to reach you, but feel free to call back and schedule at your earliest convenience. Again, our number is 503-297-3660 option 2 for scheduling. Thank you!

After Scheduling:

- In person appts:
  - Provide office address
  - Confirm appointment policies (no children in US room)
  - Check in early/bring photo ID and insurance card if new patient
  - Full bladder (if needed-before 24wks)
- Telehealth appts:
  - Confirm phone number
  - "Your provider will send you a text message with the link to sign in to your visit right at the start time of your appointment. If you do not get the link within ten minutes of your appointment time, please call the office." The link we send is a shareable link, if there is someone you would like to join with you that is not physically with you, you can forward the link to them and they can also click link to join.
  - If patient is new, send MyHealth activation link and inform patient they will need to fill out their new patient forms in their e-check in for their appt. These will need to be completed before their appt or they may have to reschedule their appt.

Ultrasound Scripting:

Scheduling an Ultrasound Appointment

"You can bring one healthy "adult" support person to your appointment." We ask that children do not attend Ultrasound appointments.

If the patient then responds, "I have to bring my child(ren) to my Ultrasounds appointment", reply using this scripting "I will let our Ultrasound Coordinator know and they will follow up with you to discuss this further." Staff Message the Ultrasound Coordinators (at WHA US Coordinators Pool) with "CHILD at visit" in the subject line and "FYI Child(ren) attending Ultrasound visit on xx/xx/xx (date)".

Ultrasound Prep Scripting

OB Ultrasound Exams Script – For Patients 24 wks or less

Your provider has ordered an ultrasound for you. In preparation to provide best imaging, since you are less than 24 wks in gestational age, please finish drinking 24 ounces of water 30 min before your scheduled appointment.

Patients over 24 wks gestational age do not have to arrive with full bladders

GYN Ultrasound Exams Script

Your provider has ordered an ultrasound for you. In preparation to provide best imaging, please finish drinking 24 ounces of water 45 min before your scheduled appointment. Please DO NOT empty your bladder after front office check in until instructed to do so by the ultrasound tech.

Pediatric Cardiology

A Pediatric Cardiologist comes to both of our offices during the month.

NWP West = 1st and 3rd Thursday of each month

NWP East = 4th Tuesday of each month

There are blocked Ultrasound appointments and MFM appointments on Epic labeled "*Peds Cardio*". These are specifically built for the Peds Cardio appointments.

**Schedule from the LAST Peds Cardiology spot available on the schedule towards the middle of the day. Appointment length = 60 min (unless otherwise indicated)

Epic visit type = Peds Cardio

** Schedules have been built for the Pediatric Cardiologist in Epic. The Pediatric Cardiologist seeing patients that day will have time scheduled within the Ultrasound time to meet with the patient.

Patients referred to Pediatric Cardiology have usually already had an initial Fetal Echo with NWP and have seen an MFM. They were referred to Pediatric Cardiology as they have seen some type of heart abnormality in the fetal heart.

How to let Pediatric Cardiology office know a patient has been scheduled at NWP:

Contact Information:

Coordinator = Tami

Phone: 503-280-3418 Ext 117

1. Send a cover sheet with patient information. This cover sheet can be found within the WHA Shared Files/NWP/Front Office. Pediatric Cardiology Referral
2. Print the patients AMB Face sheet in the patients' chart under the "SnapShot" tab

3. Print of copy from Epic of the patient's most recent ultrasound report
4. With the cover sheet on the top, fax all pages to the fax number on the cover sheet
5. File these pages in the file box on the counter, in alphabetical order
6. Please fax these documents for ***every*** Peds Cardio appointment scheduled (Even if patient has been seen by Peds Cardio already and the new appointment scheduled is a Follow Up appointment.)
    - Melissa confirms with Peds Cardiology office the schedules match one week before and two days prior.

Pediatric Cardiology Coordination Review Process

1. One week prior to appointments: Review Ultrasound Schedules within Epic for the scheduled day
    - 1st and 3rd Thursday for NWPW (US Room 2 &3)
    - 4th Tuesday for NWPE (US room 1)
2. Call Pediatric Cardiology **See contact information above
        a. Review schedules with contact at Peds Cardiology to make sure patients are matching
            i. Example 1205 is patient Mary Smith
3. Two days prior to appointments review schedules again and call Peds Cardio to confirm appointments are still matching.
4. If there are cancellations or add-ons, please let them know

**If you need to schedule an urgent appointment to be with Pediatric Cardiology, a phone call to Pediatric Cardiology office (Tami) will need to be made before adding another patient to the schedule for approval.

Pediatric Urology Coordination of Referrals sent for scheduling

New process to be added when information becomes available.

In Basket Workflow

Best practices for in basket management calling include:

- Avoid 'cherry picking' – this means individuals should take the baton in order from higher priority to lower priority. When higher priority is completed, then working from oldest to newest so patients are contacted in a timely manner.
    - Should not avoid working on an item due to higher complexity (example – needs interpreter, multiple appointments/ancillary services needed, etc)
    - Exceptions to this would be if you are at check-in and jumping in between appointments. It is not ideal to be on a long call while at check-in if avoidable.

- Take the baton when you are working on an item. Do not take the baton unless you are actively working on the item. Do not take multiple items if you are not going to be able to work on the item within 30min of taking the item

- Remove the baton if something happens where you will not be able to address the item within 30min

- Document everything in the summary as defined in our workflows

- Update the Comments after work has been done so the status can easily be seen by others

Scheduling

Holding Times for Appointments

The patient services team will add holds onto the schedule (provider, ultrasound, NST, and other resources needed) and also notate in the NWP Patient Summary.

Holds should have the **date the hold was added, MRN and the full name of the team member adding the hold**. Example: 9/14/26/MRN ***/Lindsay Weinrich

Holds should be added for the following visits:

- Urgent appointments – Defined as needed to be scheduled within 2 weeks. This can include established, new, and/or procedures
    - Add holds in the 2-week time period as defined by provider

- Amnio, CVS, KCL procedures
    - Add holds in the time defined by provider

- Established patients – Defined as patients who have had a MOD/MFM consult for this pregnancy. Team to add holds 4 weeks out for these patients
    - Add holds up to 4 weeks out as defined by the provider

- Consults – Defined as patients who have not had a MOD/MFM consult for this pregnancy. Patient may or may not be established at WHA
    - Add holds up to 6 weeks out as defined by the provider

Removing Holds

Holds should be removed promptly and documented in the Summary for the following reasons:

- Patient accepted hold times – Remove holds, schedule patient, update the summary

- Patient declined hold times – Schedule as agreed to with patient, remove holds, and update the summary

- Two or more call attempts to the patient – Release holds and update the summary

Note: we should attempt to contact the patient two times via phone and one time via MyHealth - totaling 3 attempts. If the patient does not follow up, release holds and send to referring provider for awareness

- Cancelled referral – Remove holds and update the summary

- Declined scheduling – Remove holds, update the summary, and notify referring/ordering provider

Scheduling Reminders

- NST Scheduling – Must schedule NST when a provider is scheduled to be physically in clinic
- 7:20am Ultrasounds – Must be Growth, Anatomy, or GYN. **Cannot** be D&V
- West Ultrasound Scheduling Rooms 2 & 3 – West rooms 2 & 3 are our sono 2 team members. It is okay to schedule Growths, Anatomy, D&V in other West rooms to preserve sono 2 schedule for appropriate scans

Genetics Scheduling

Note: Non-AMA (under the age of 35 at time of EDD)

Non-AMA patients can be seen by a Genetic Counselor if they have a history of or Family History of a Genetic disorder. These cases MUST be reviewed by the CCC (Clinical Care Coordinator) and/or Genetic Counselors prior to scheduling.

Note: AMA Testing Options

Patients are considered AMA if they are going to be 35 at the time of delivery. However, patients with Twin pregnancy are considered to be AMA if they are going to be 33 years old at the time of delivery.

- **NIPT**

** NIPT (non-invasive prenatal testing also known as Cell Free Fetal DNA, Verifi, Harmony) – offered to non-AMA and AMA patients

How to Schedule:

Non-AMA: Test includes a blood draw and can be scheduled at their WHA doctor's office (WHA is offering NIPT to our low-risk population of under 35 at the time of delivery. Some external OB providers are referring to NWP who are under 35 for NIPT. These patients MUST meet with a Genetic Counselor prior to the lab draw.

AMA: Test includes a 60min Genetic Consult, and lab draws done after 10 weeks and has no- cutoff date.

- **Sequential Screening (not ordered often)**

**Sequential Screening – Offered to non-AMA and AMA patients.

How to Schedule

The sequential screen is a 2 part test. SEQ 1 is done between 11.3wks to 13.6wks. However, 12 weeks is the most recommended time frame to start scheduling.

Non-AMA: Test include 50min US and lab draw

AMA: Test includes 60min Genetic Consult, 40min US, FYI to US Reader (SEQ 1 AMA apt type), and lab draw.

When to schedule PART 2 of SEQ Screening: SEQ 2 is between the optimal time frame of 16-18 weeks. It can also be done between 15-22wks. Patient should have a piece of paper from the lab with the best time frame for them to schedule the SEQ 2. SEQ 2 consists of just a blood draw on the lab schedule. (This can be scheduled at any WHA office if patient desires)

- **Chorionic Villus Sampling (CVS)**

**Chorionic Villus Sampling (CVS) – offered to AMA and non-AMA patient with a history of genetic disorder.

How to Schedule

Scheduled between 11.3wks -13.6wks. The Test consists of patient meeting with a Genetic Counselor, Ultrasound and a 40 min MFM and nurse and a lab draw. Specific MFM are able to complete the CVS, reach out to your Patient Services Supervisor for guidance

- **Amniocentesis (Amnio)**

**Amniocentesis (Amnio) –Offered to AMA or non-AMA patient with History of genetic disorder.

How to Schedule

Usually scheduled around *16wks of pregnancy – optimal time frame 15-18wks. Test consists of Genetic Consult, Ultrasound and last 20min of US with a MFM for the Amnio procedure on US Reader schedule for both East and West.

If a patient is scheduled to do a possible Amnio and for some reason we cannot complete the Amnio procedure at that appointment, the patient only needs to come back to meet with the genetic counselor for a brief 30 min and an Ultrasound with Amnio.

1. Genetic Consult for Testing Options Counseling - schedule 60min starting around 10wks
2. Genetic Consult for Reviewing Test Results - schedule 60min next available
3. Genetic Consult *"consents only" prior to procedure - schedule 30min on day of or day before procedure. *Consents only = the patient has already had Genetic Counseling can be 30min and No FYI needed.

Notes

- Patients can only complete one Genetic test. They can do another test if the 1st genetic test comes back with an increased risk.
- If a patient has already met with a Genetic Counselor and is scheduling later to return for Amnio, the patient will need to see a Genetic Counselor for 30min prior to the US/Amnio to sign consent forms

Example

- Non AMA patient completes SEQ 1 and has an increased risk for Down syndrome. This patient will come back in to speak to a genetic counselor and possibly do further testing with NIPT, CVS or Amnio.
- Most AMA patient will 1st come in to discuss testing options and then schedule to come back for the option that they opt for.

- **Scheduling for Increase Risk from SEQ Screening/Quad Screening or NIPT**

Default scheduling should be (unless instructed differently) if the patient is at least 15 0/7 weeks at the time of the Consult: Genetic Consult, Ultrasound, option for Amnio/Blood draw.

No Call NIPT **This means no results came back from the NIPT for some reason Ex: Low Fetal Fraction

Ultrasound and Genetic Counselor

Counsel Testing

All patients and partners need to meet with a Genetic Counselor prior to having the lab drawn so the counselor can order the correct panel to be drawn.

- **KCL Injection Scheduling**

Scheduling consists of an Ultrasound, MFM Provider and Nurse Resource (and possibly Nitrous Oxide equipment)

Ultrasound 60min (using Amnio visit type)/Provider/Nurse all together for 60min

Schedule as follows:

- Amnio only visit type
- Select Ultrasound room
- Add NWP Nurse schedule US room/resource list
- Select both Nurse and Ultrasound schedule and check the "joint appt" button
- Select Provider
- Change Amnio Ultrasound and Nurse visit length to 60min
- Select times on the schedules

*Send staff message to advice pool for awareness of the upcoming KCL injection procedure that is scheduled.

Laminaria Visits:

This visit type is scheduled as an OB Check visit type, RN for 15 mins, Provider for 25 mins (and possibly Nitrous Oxide equipment)

Schedule as follows:

- OB Check NWP visit type

Laminaria placement procedures at NWPW - the CCC will place a hold on the **PKN's Procedure** Room schedule

Room A stating "NWPW Lam Placement" for the duration needed as long as the room is available.

Patient Services: If you see that the **PKN Procedure Room A** has been booked in the scheduling notes within the Patient Summary. **Please add to the appointment notes "OB visit for laminaria in PKN Procedure Room A"** as the room will not be linked to the appointment.

Cerclage Removal Visits:

This visit type is scheduled as an OB Check visit type, RN for 15 mins, Provider for 25 mins (and possibly Nitrous Oxide equipment)

- OB Check NWP visit type

Cerclage Removal w/MFM Name **PKN Procedure Room A held for procedure

On schedule hold PKN Procedure Room A for 40 mis (15 min RN/25 min MFM) *Note in Hold: Date, Patients Name, MR#, Holding for Cerclage Removal w/MFM Name//Your first name and last initial*

Prior Authorization for CVS and Amniocentesis

1. Scheduler needs to send a Staff Message to the NWP Financial Counselor Pool to notify that a prior authorization needs to be started. The Staff Message should include type of procedure, date of appointment and insurance.
2. Scheduler will make note in the **Patient Summary** that a Staff Message has been sent to FC for prior authorization
3. Financial Coordinator will review the Staff Message and starts prior authorization paperwork process.

ALL URGENT PRIOR AUTHORIZATINS (LESS THAN 24HRS) CONTACT a NWP Financial Coordinator via Phone due to its Urgency

Postpartum Delivery Notification & Scheduling

- Send a staff message to the NWP Advice pool anytime you learn that a patient has delivered (for example, when canceling upcoming appointments due to delivery)

- In addition, send a post-dated staff message whenever PSR schedules an induction or C-section to support timely follow-up

    - Use the following documentation: **Patient delivered on **. Please call patient to complete 1-week PP call. If already received notification, please disregard it.**

Our West and East OB coordinators will continue their workflow of calling hospitals for delivery reports and sending a staff message/telephone encounter to complete the 1-week postpartum call. Because hospital lists are not always complete, this additional workflow will help minimize care gaps.

Postpartum NWP for the 3 week PP MFM Visit

POSTPARTUM NWP (POSTPARTUM) 2436

Appointment Notes: of weeks PP, delivery date - Example: 3 week PP, delivered on 04/12/2025

WHY this change:

- The Postpartum NWP visit type has the EPDS (depression screening scale questionnaire) attached and it only needs to be filled out at the routine PP3

Postpartum Triage NWP for all other MFM and MOD postpartum visits other than the routine 3 week visit: this includes Blood pressure checks, Postpartum concerns, bleeding, depression, etc

POSTPARTUM TRIAGE NWP (POST PARTUM) 3720

Appointment Notes: # of weeks PP, delivery date - Example: 1 week PP, Blood Pressure check, delivered on 04/12/2025

WHY this change:

- The Postpartum Triage NWP visit type **Does NOT** has the EPDS (depression screening scale questionnaire) attached to it

Notes: When using the Postpartum Triage NWP for **MOD Patients,** you will need to pull in and select the MOD provider and change the time to 30min as it defaults to 25min.

MOD RISK Postpartum for the 3 week PP MOD Visit

MOD RISK POSTPARTUM NWP (POSTPARTUM) 3050

Appointment Notes: # of weeks PP, delivery date - Example: 3 week PP, delivered on 04/12/2018

WHY this change:

- The MOD RISK Postpartum NWP visit type has the EPDS (depression screening scale questionnaire) attached to it and it only needs to be filled out at the routine PP3

Please continue to use the **PP telehealth visit NWP and the Mod Risk telehealth NWP for any video visits no matter the postpartum reason

Tips for Routine 3-week postpartum visits:

1. Schedule between 21-56 days after delivery. Delivery day=Day 0 and count days after
   a. Scheduled with either the primary responsible provider or the delivering provider
   b. Not to be scheduled with anyone else who hasn't worked with the patient. If a patient no shows her 3-week postpartum visit and attempts to reschedule, she should only be rescheduled with their provider or delivering provider.
2. BP checks and hospital follow-up
   a. Follow-up arranged for non-routine postpartum visits such as BP checks, early incision checks, diabetes follow-up, etc., these can be scheduled with any of the MFMs as these tend to be more time dependent. However, these patients should be seen by either their primary provider, delivering provider, or discharging provider if possible.

How to find the patient's delivery date/discharge summary in Epic

- Pull up the patient's chart and click on the Care Everywhere icon in the upper left side of the screen near the patient's picture
- If the date shown next to Providence Documents is not up to date, click on "request updates"
  - Listed under **Care Everywhere Outside Records**) then click "documents"
- Click "Admission" on the left-hand side under "Type"
  - In the upper right-hand corner of the screen, hover over "Jump to Section"
- Find "Discharge Summaries"
- Admission date may not be the same as delivery date, so look/read into the notes to find delivery date

**Patients of NWP usually come to NWP for the 1 week and 3-week Postpartum appointments. In most cases, patients are to go back to their primary OB/GYN doctor for the 8-week Postpartum visit, unless otherwise indicated in the scheduling instructions or hospital Discharge Summary notes.

Registering & Scheduling Patient Partners for Lab Draws

Background:

When a patient and/or partner are scheduled for a lab draw the Patient Services Staff standard workflow is to leave the Appointment Notes section blank. The lab assistants add the appointment notes after reviewing the schedules/orders for the patients scheduled. When scheduling partners, include the NWP's patient MR # in the partners NWP Summary.

Workflow Summary:

When a patient's partner is scheduled for a lab draw, we will now be launching a Patient Summary from within the partner's chart. *(**Not** in the patient's chart)*

Schedulers Workflow

1. NWP PS Team members will be notified by either Genetic Counselor or MFM that a patient's partner will need to be registered and scheduled for a lab draw.
   a. These scheduling instructions will come from the patient's chart by a message to our Patient Services pool or in-person while in clinic, as the partner usually does not have a chart created in Epic.
      - If the patient's partner calls in and you do not see any notification, please reach out to a Genetic Counselor for what is being ordered and when to schedule. Since the partner is not registered, most of the initial communication and instructions will be in the patient's chart notes or Patient Summary.
2. The patient's partner will need to be fully registered in Epic.
   a. If partner is in-office, fully register and complete all necessary new patient paperwork, including Permission to Communicate, scan Driver's License and Insurance card(s).
   b. If partner is calling in by phone, fully register and complete all standard patient registration fields. ***When registering please make sure partner's insurance is one we are contracted with. If insurance is not contacted, please reach out to the admin team.*
      Upon Check In, please complete all necessary paperwork, including Permission to Communicate, scan Driver's License and Insurance card(s).
3. Follow the scheduling instructions schedule partner for lab draw at NWP East and NWP West locations only. **If scheduling at NWP East, please add "NWP Pt" to the visit notes
4. Routing Patient Summary
   - For Genetic partner lab draws route to Genetic Counselor Pool after scheduling
   - For Perinatal partner lab draws route to Care Coordinator Pool after scheduling

Pain Management Options for NWP Patients

Background: WHA/NWP now offers Nitrous Oxide via the Pronox machine to our patients for anxiety and pain management for visits.

FYIs:

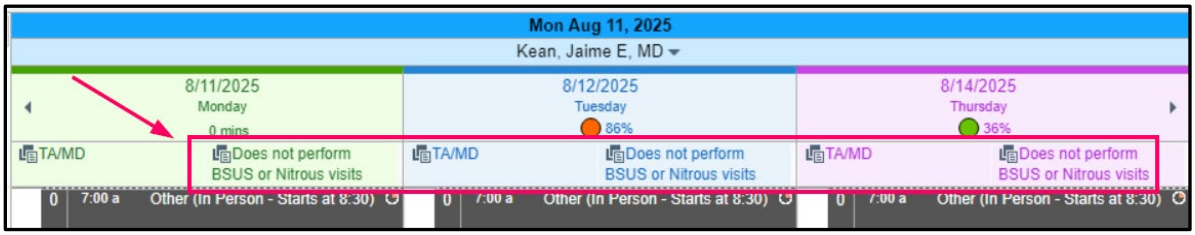
- NWP shares the Nitrous resource, PKN Procedure Room A, within Epic with the Peterkort office. This is the resource to use when needing to book Nitrous
- Patients must pay a fee at check in or check out for their Nitrous Oxide assisted visit ($100 for Commercial insurance patients, $30 for Medicaid / Medicare patients and $100 for Self-Pay patients)
- If patients have Medicare as primary or secondary, it will still only be $30 (even if they have a Supplemental plan)
- Add message to all visit appt notes if you do book the Nitrous Resource/**PKN Procedure Room A** so all teams at NWP can confirm its reservation

Which Providers can be scheduled with Nitrous Oxide
Collecting the Nitrous Oxide Fee (Check-In / Out Notes)
Scheduling Nitrous - Cerclage Removal
Laminaria Placement/KCL Procedure Overview
How to add the Nitrous resource to a visit

Which Providers can be currently booked with Nitrous Oxide?

- **NWP** – All Providers – Except Dr. Smrtka (and some US rooms – see Epic template for note)

If the Provider has opted out, you should see a Template note like this one above their schedule in Epic:



Collecting Nitrous Oxide Fee

- Patients must pay a fee at check in or check out for their Nitrous Oxide assisted visit ($100 for Commercial insurance patients, $30 for Medicaid / Medicare patients and $100 for Self-Pay patients). If patients have Medicare as primary or secondary, it will still only be $30 (even if they have a Supplemental plan).

- The Patient Services team is able to collect a Nitrous Oxide fee without our Financial Team posting it prior. See the chapter titled Payment Categories / Payments – Adding a Prepay in the Check-In Manual for steps.

  This includes taking payments outside of Check-In. For example, when a Nurse

walks the patient (who has already started the rooming process) back to the front desk to pay their Nitrous Oxide fee before starting their visit with the Provider.

- If a payment for a Nitrous Oxide fee is taken, Staff Message the Billing and Coding team to let them know so that they can apply the payment appropriately.

Collecting fee during Check-In / Check-Out

For patients paying at Check-In, the prepay fee could already be listed on the patient's account to apply the payment to.

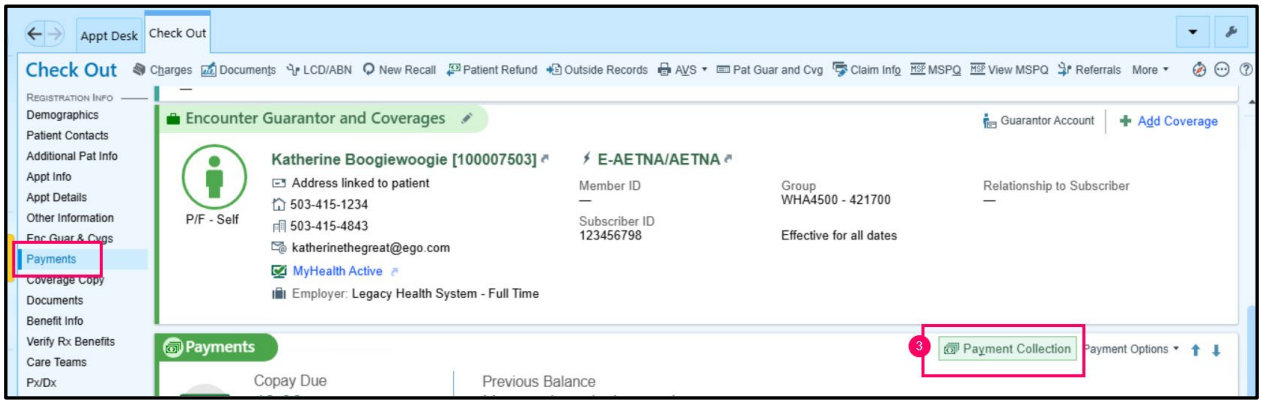
If you know the patient is getting Nitrous Oxide today and a prepayment is not listed, follow **steps 2 – 4** under the Check-Out process below:

At **Check-Out**, patients may come back to the front desk after their visit with an orange laminate Nitrous Oxide card. When they do:

1. Collect the orange card 

2. Verify what type of insurance the patient has in order to determine how much their fee will be ($100 for Commercial insurance patients, $30 for Medicaid / Medicare patients and $100 for Self-Pay patients)

3. Use the Payment Collection tool in Epic or within the Payments section of the Check Out or Check In window, add a **PREPAY OTHER** with a comment that this is for same day Nitrous Oxide



4. Collect the appropriate fee amount from the patient
   - If you are collecting the $100 or $30 fee **during check-in**, use the Prepay listing of **OTHER AR**

   - If collecting the $100 or $30 fee **during check-out**, use the Prepay listing of **PREPAY OTHER**

(more on next page)

Scheduling Nitrous - Cerclage Removal

For this visit reason, Nitrous is usually added to the patient's 32-week OB Check with their MFM. To add Nitrous to an OB Check use Reschedule rather than "Change Appt" in Epic due to possible financial and templating issues

Overall Cerclage Workflow:

1. Patient has cerclage placed in the hospital, is seen with a MFM for a "postop visit" in clinic
2. At that visit, MFM may review pain management options for planned removal (if done in clinic)

Note: If patient is consulted and chooses Nitrous, the Clinician or the CCC will send a note to the NWP PS team via the Patient Summary instructing them to add the Nitrous Resource to the 36-week OB Check with the MFM

3. If unable to review at postop visit, MFM will send PS team instructions to schedule a MFM Telehealth visit at 32 weeks to consult procedure and pain management options with the provider *(if patient is not already scheduled to be seen by MFM between postop visit and removal at 36 weeks)*.
   If the patient does get scheduled for the consult visit (MFM Telehealth visit), the NWP PS Scheduler will send the patient a Myhealth message of Pain Management options using the dot phrase - **.WHAPROCPAINMGMTCOMM**

Note: If the MFM scheduled for the removal is not available for the pain management consult then it is ok to schedule with any available MFM to review options, including Nitrous, with the patient

Scheduling Nitrous - Laminaria Placement/KCL procedure

Note: The number of times the patient may pay for nitrous based on lams/KCL

Once instructed by a Clinician or the CCC, add Nitrous to the OB Check or Amnio Procedure visit type

Laminaria Placement/KCL Procedure Overview:

1. Patient requires laminaria placement and/or KCL and consult is completed with WHA clinician (pain management reviewed at that visit)

2. Surgery Scheduler discusses pain management/Nitrous while scheduling the D&E procedure and if the patient expresses interest in Nitrous, they will notify the provider who is scheduled to do the initial consult

   Note: After the initial consult, if the patient requests Nitrous, **a Staff Message is sent by the Surgery Scheduler** to the NWP Financial Coordinator Pool to arrange payment (if lams is planned with their provider)

3. Surgery Scheduler notifies CCC of plan/next steps if MFM is involved, and CCC sends scheduling instructions to NWP PS team to schedule

   - If patient was counseled on Nitrous by another WHA/MFM provider different from the provider performing the procedure, ok to proceed with Nitrous *but Counseling must take place to proceed with scheduling*

   - NWP PS team sends pain management information via Myhealth patient messaging using the dot phrase - **.WHAPROCPAINMGMTCOMM**

Note: Make sure the message settings are set so the patient **does not** reply back directly to you



We want you to feel supported and prepared for your visit. Please read the information below about pain management options and call us before your procedure to talk through your options and make a plan. Depending on your situation, we may schedule time for you to speak with your provider or connect you with a nurse to help set things up. Some options take extra time to arrange, so the sooner you reach out, the better. We're here to help make your experience as comfortable as possible.

Pain Control for OBGYN Procedures in the Office | What You Should Know

At WHA, we understand that many people find OBGYN visits and procedures uncomfortable. This can include physical exams, Pap smears, injections, blood draws, ultrasounds and mammograms. Some procedures, like inserting an IUD or taking a biopsy, can be painful. That's why we offer options to make you more comfortable.

Why Are OBGYN Procedures Sometimes Painful?

- **Sensitive areas:** The vulva, vagina, cervix and uterus are very sensitive. Just like touch in these areas can be pleasurable, it can also cause pain during procedures. The uterus, in particular, may cramp when touched, making IUD insertions or endometrial biopsies uncomfortable.
- **Past experiences:** If you've had past trauma or existing pain, you might be more sensitive to touch, which can make some procedures harder to tolerate.
- **Anxiety:** Even without prior trauma, feeling nervous during gynecology visits is common, and it can increase the feeling of pain.
- **Your anatomy:** Everyone's body is different. Sometimes, it's harder for a clinician to see inside the vagina or examine the cervix because of the unique size or positioning of your organs.

Everyone feels pain differently. If you're experiencing too much pain during your care, let your clinician know so they can adjust the plan. At WHA, we're committed to your safety and comfort, and no one will be upset if a change is needed to help you feel more comfortable.

What Options Are There to Manage Pain During Gynecology Procedures?

- **Numbing medicine:** Like at the dentist, your clinician can use numbing medicine (like lidocaine) to reduce pain. This helps block sharp pain but you may still feel touch or pressure, and it doesn't prevent cramping.
- **Anti-anxiety medicine:** Your clinician can prescribe medication to help reduce anxiety before a procedure which can help you relax and may lower your sensitivity and perception of pain.
- **Pain relievers:** Over-the-counter pain relievers like ibuprofen, naproxen, or Tylenol taken before a procedure can help with discomfort afterward. Your clinician may instead choose to prescribe a dose of pre-procedure Toradol (20 mg) to take 60 minutes before your procedure.
- **Opioid medication:** Medications like tramadol or oxycodone can help with pain or cramping but may cause side effects like drowsiness or nausea and you may need someone to drive you to and from your appointment.
- **Nitrous oxide:** Sometimes called "laughing gas," this inhaled gas helps reduce anxiety and pain during procedures. It can be used along with numbing medicine.
- **In-Office Surgery at WHA:** For some procedures, IV sedation provided by a specialized nurse in one of our offices is an option. This is similar to what some people have for a colonoscopy or wisdom teeth removal. It may require special scheduling and could be more costly. You will need someone to drive you home.
- **Hospital anesthesia:** For some cases, full sedation at the hospital may be best, even for routine procedures like a pelvic exam. If you and your clinician decide this is the right option, it's always available.

Schedule Your Procedure with Someone You Know

If you can, try to schedule your procedure with a provider you've seen before and feel comfortable with. This can help reduce any anxiety and make the experience easier. Let our scheduling team know if you'd like to stick with your regular provider.

Let Us Know What You Need

We understand that everyone has different needs when it comes to managing pain. Things like transportation, cost, and time can all play a role in deciding what pain relief options work best for you. Please share your needs with us so we can create a plan that fits your situation.

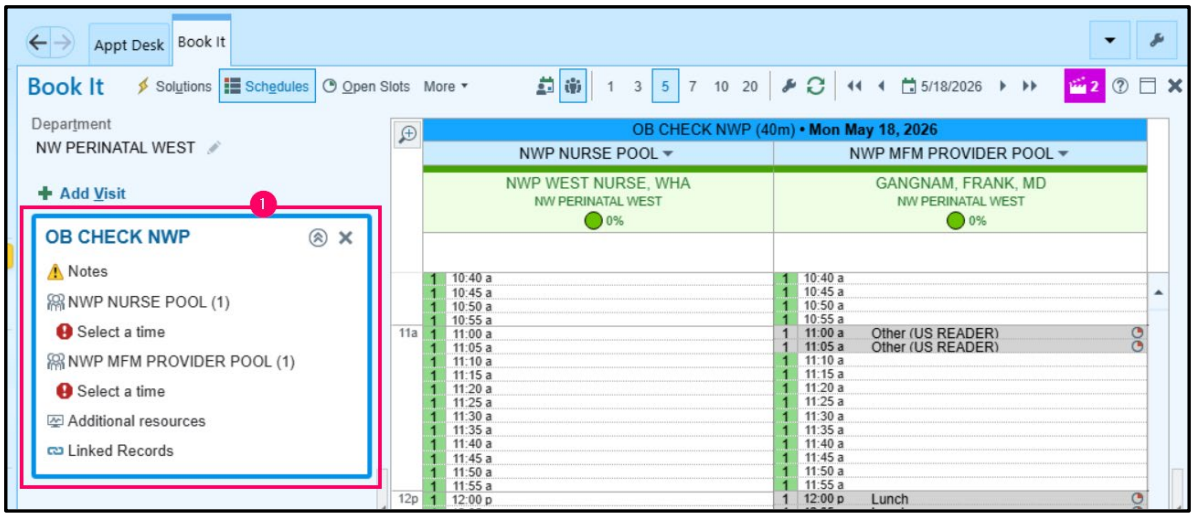
Rev 6/2025

- 25 min Telehealth MFM visit is scheduled prior to KCL procedure, this visit can also include reviewing pain management/Nitrous options

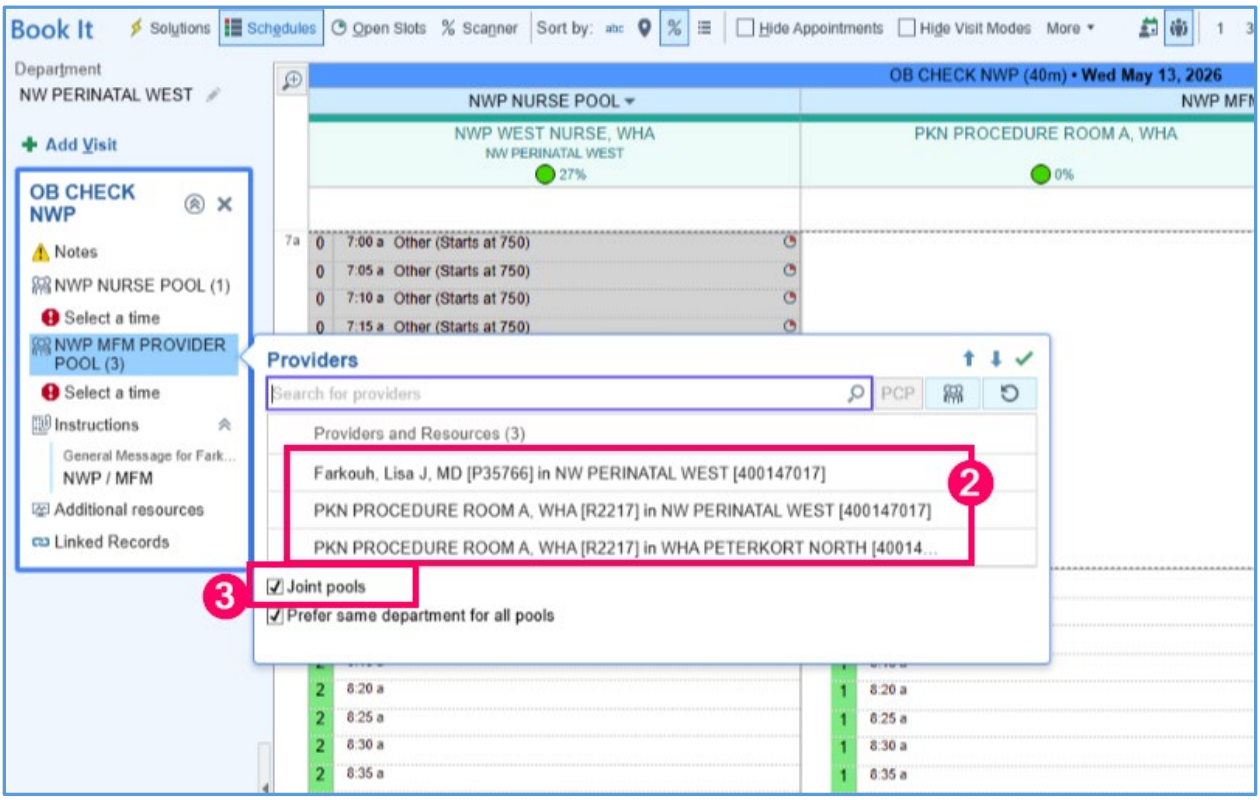
How to add the Nitrous Resource to a visit

To schedule a **new visit** with Nitrous Resource, click Book It from the Appt Desk, then follow the steps below -

To add the Nitrous Resource to an already existing visit, click Cancel/Reschedule, choose **Reschedule** then follow the steps below -



1. Set up your Visit Card with the Visit type you are wanting to schedule and choosing your Nurse and MFM(s)

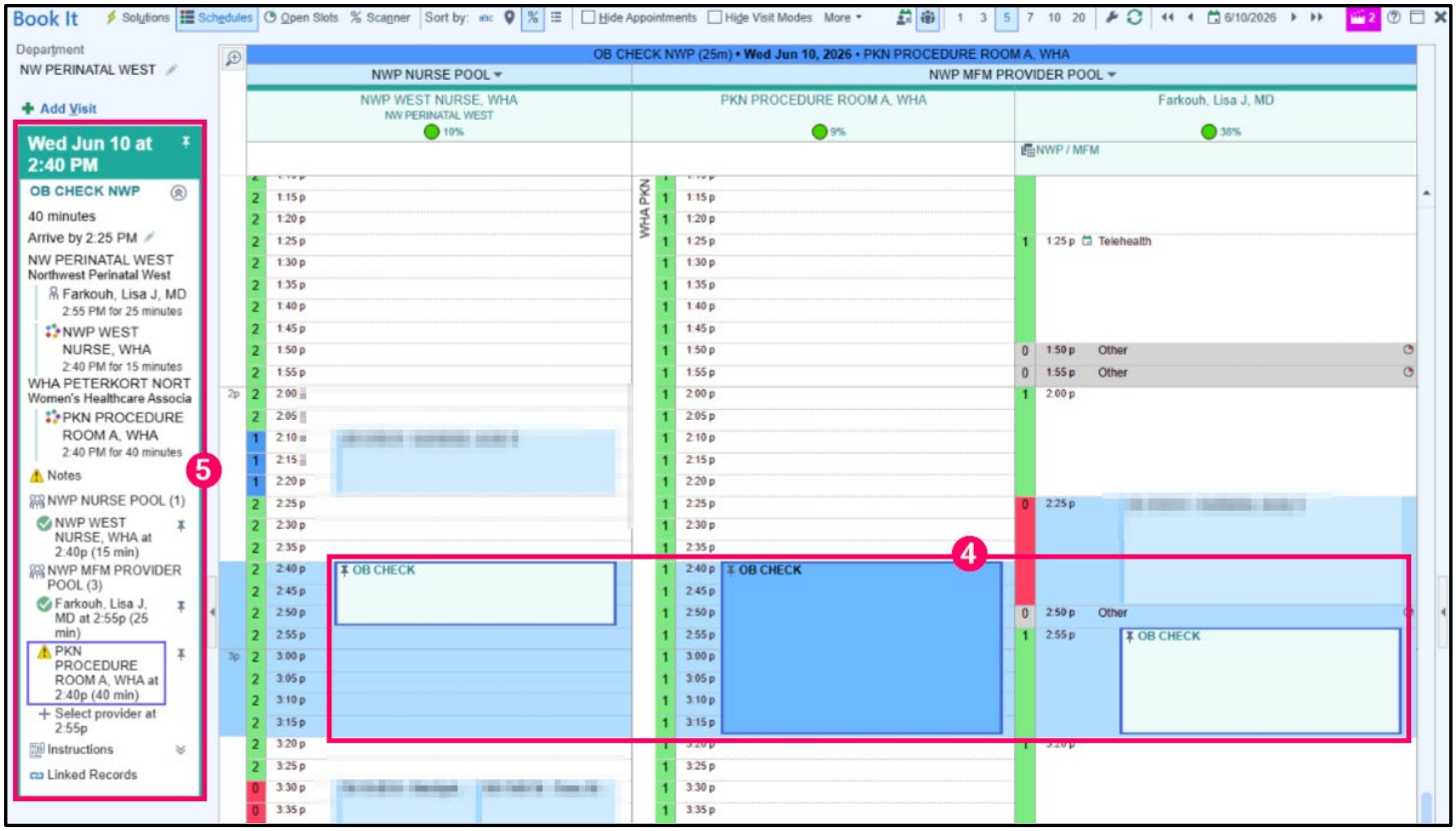


2. Click on the MFM Provider selection field and within the search box type/add:

PKN Procedure Room A

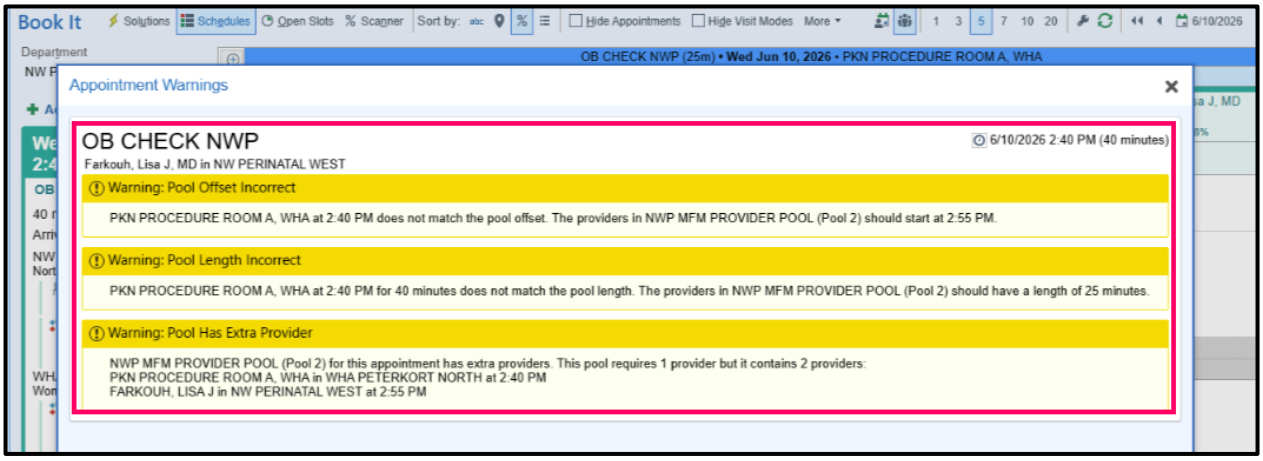
Note: NWP shares the Nitrous machine with PKN and that resource within Epic is called PKN Procedure Room A

3. Click the Joint Pools box



4. It is important to book the PKN Procedure Room A resource for the entirety of the patient's visit that the machine is available the entire visit for the Clinician

5. The Nurse, MFM, and Nitrous machine (Procedure Room A) should be within one Visit Card – *there might be an Ultrasound room also selected if you are scheduling an Amnio Procedure*



You might get this warning after you click Schedule. This is happening because you added an extra Provider/Resource (the Nitrous Resource), but it is okay and you can click Continue past it

MFM Scheduling at Gateway

Background:

Implement proactive scheduling for Gateway referred patients to improve access and reduce delays by allowing appointments to be scheduled prior to referral entry and formal clinical review, based on MFM guidance. Final referral submission and clinical review will occur after scheduling and may result in appointment adjustments.

Workflow:

1. Care Plan Discussion
    - Gateway provider discusses the patient plan with MFM
    - MFM provides scheduling guidance (e.g., timing, NST, ultrasound)
2. Scheduling (Based on NWP Presence at GW)

    A. When NWP is Onsite at GW:

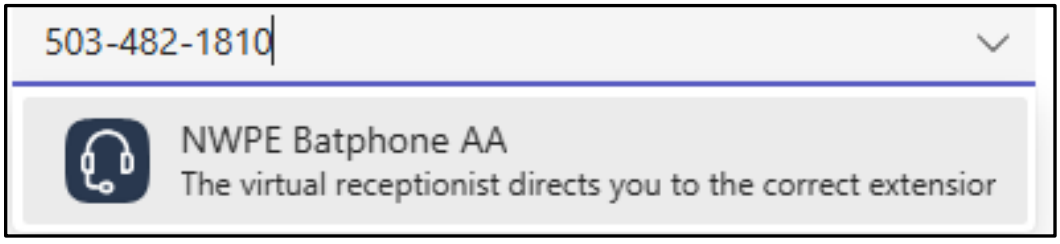
    - GW provider/PSR coordinates directly with the onsite NWP PSR
    - GW Provider/PSR relays MFM guidance for scheduling to NWP PSR
    - NWP PSR schedules the patient accordingly
    - NWP PSR creates a patient summary and routes to CCC pool noting if waiting on referral entry (see below for further details)

    B. When NWP is NOT Onsite at GW:

    - GW PSR contacts the NWPE Bat line
    - GW PSR relays MFM guidance to the NWP PSR
    - NWP PSR schedules the patient accordingly
    - NWP PSR creates a patient summary and routes to CCC pool noting if waiting on referral entry (see below for further details)

3. Proactive Scheduling (Prior to Referral Entry)
    - Appointment is scheduled before referral is formally entered
    - Scheduling is based on MFM guidance and patient availability
4. Documentation & Communication
    - NWP PSR:
        - Creates a patient summary
        - Routes to the CCC pool indicating:
            - Patient is already scheduled
            - Referral is pending and not yet entered or under review
    - Patient is informed:
        - Appointment is scheduled
        - It may be adjusted after referral review
5. Referral Submission (May Occur After Scheduling)
    - GW provider submits the referral
6. Clinical Review (Standard CCC Process)
    - CCC completes referral review once received
7. Post-Review Outcome
    - If appropriate: Appointment remains as scheduled
    - If changes are needed: Appointment is modified accordingly

The NWP Bat line:



Pacific Northwest Fertility Ultrasound Patients

Background: NWP will be accepting ultrasound orders for patients from Pacific Northwest Fertility. Pacific Northwest Fertility is a reproductive endocrinology and infertility (REI) clinic who helps support patients who are needing a higher level of care in order to become pregnant. While Pacific Northwest Fertility primarily sees patients in Seattle and Bellevue Washington, they have a subset of patients who are seen closer to the Portland area, we want to help support these patients with ultrasound and lab needs. These patients can be scheduled for GYN ultrasounds or early OB viability ultrasounds and labs.

Note: *The USC team will be the primary team supporting scheduling these patients*

Chapters:

FYIs: Visit Types to use / Check – In / Reschedules
Locations / Provider List
Contact Information
Overall Workflow

FYIs

These patients will be scheduled with specific visit types for tracking and so it is easy to determine when it is a REI patient:

- **REI GYN US WHA/NWP** – 60 mins
- **REI OB US WHA/NWP** – 50 mins
- **REI LAB WHA/NWP** – same default length as our other lab visit types

During the check-in process you will go over ***all patient consents*** as normal - you do not need to give these patients the WBA

If there are reschedules, the USC team will support rescheduling the patients. We have created specific blocks on the ultrasound schedules to accommodate these patients. Our

ask if it is an early morning ultrasound, that we are notifying the patient of the cancellation and the USC team will reach out to them to reschedule.

Locations

Seattle Office
1101 Maddison St., Suite 1050
Seattle, WA 98104

Bellevue Office
1231 116th Ave NE, Suite 750
Bellevue, WA 98004

PNWF Providers:

- Shannel R. Adams, MD 1235455403
- Darcy E. Broughton, MD 1386934925
- Hanh N. Cottrell, MD 1902122385 (leaving in June 2026)
- Julie D. A. Lamb, MD 1154494128
- Jamie A. Massie, MD 1255403622
- Stephanie S. Rothenberg, MD 1659647360
- Lora K. Shahine, MD 1578643524
- Shelun Tsai, MD 1760873426
- Lauren E. Verrilli, MD 1508253436
- Diana X. Zhou, MD 1235543950
- Angela DeBay, ARNP 1083487458
- Lulu A. Hong, PA-C 1376195628
- Paige A. Maberry, PA-C 1245911999
- Olivia A. Mills, ARNP 1902561376
- Jennifer R Schilling, ARNP 1760511232
- Resa A. Tucker, ARNP 1871775932

Contact Information

WHA Designated Fax Number: 503-416-7535
Faxes sent to this # will land in this folder: P:\Inbound Fax\NWP\Inbound\ReferralREI
WHA US Coordinators Phone Number: 503-296-7416
Direct PNW Fertility Manager Back Line Phone: (206) 388-4759
Direct PNW Fertility Fax Number: (206)-515-0001

Ultrasound Coordinators:
Dana Krebs
Peyton Brown

NWP Care Coordinator:
Marci Grant
Sharyce Sheets (backup)

Overall Workflow

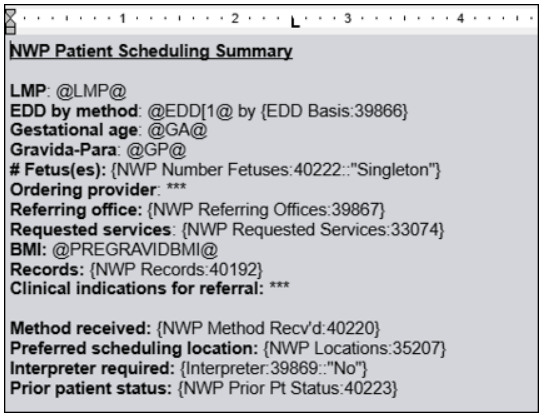
Below is the workflow if you have questions regarding the overall process –
1. PNW Fertility will fax over new patient information, records, order/request form and scheduling details directly to the USC team via designated fax #.

2. All referral documents will be available in:
P:\InboundFax\NWP\Inbound\ReferralREI

Of note, patient information may be sent prior to orders. If so, USC team will register the patient and hold on sending to CCC until orders received.

4. USC (Ultrasound Coordinator) registers patients (if not already in Epic) and updates relevant clinical information (e.g. LMP, GPs, pregnancy episodes if pregnant, etc).
5. USC sends a staff message to the Financial Coordinator NWP pool notifying them of the new patient.

6. FC team reviews coverage status utilizing benefits gathered by PNWF or identifies patient as self-pay
    - **If insured:** FC will utilize the benefits information gathered by PNWF and relay applicable benefits and estimate information to the patient.
    - If applicable, obtains authorization for services
    - **If the patient is Self-pay:** The Financial Coordinator will use the specific estimate template and send to the patient to share pricing
7. The FC team member will attach estimates to appointments for both insured and uninsured patients
8. Next the USC team creates an NWP Patient Summary encounter:
    - Reason for Doc is "Other: Comments PNWF"
    - NWPPATSCHEDSUMMARY SmartPhrase is entered and completed.



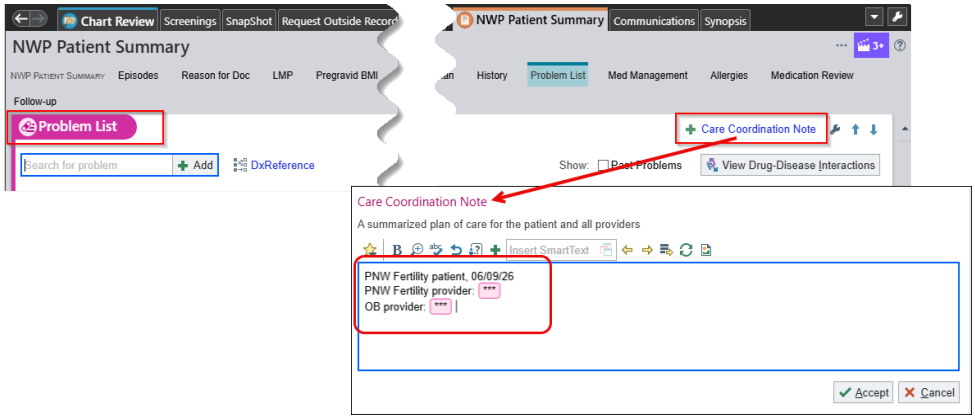
NWP Patient Scheduling Summary

LMP: @LMP@
EDD by method: @EDD[1@ by {EDD Basis:39866}
Gestational age: @GA@
Gravida-Para: @GP@
Fetus(es): {NWP Number Fetuses:40222::"Singleton"}
Ordering provider: ***
Referring office: {NWP Referring Offices:39867}
Requested services: {NWP Requested Services:33074}
BMI: @PREGRAVIDBMI@
Records: {NWP Records:40192}
Clinical indications for referral: ***

Method received: {NWP Method Recv'd:40220}
Preferred scheduling location: {NWP Locations:35207}
Interpreter required: {Interpreter:39869::"No"}
Prior patient status: {NWP Prior Pt Status:40223}

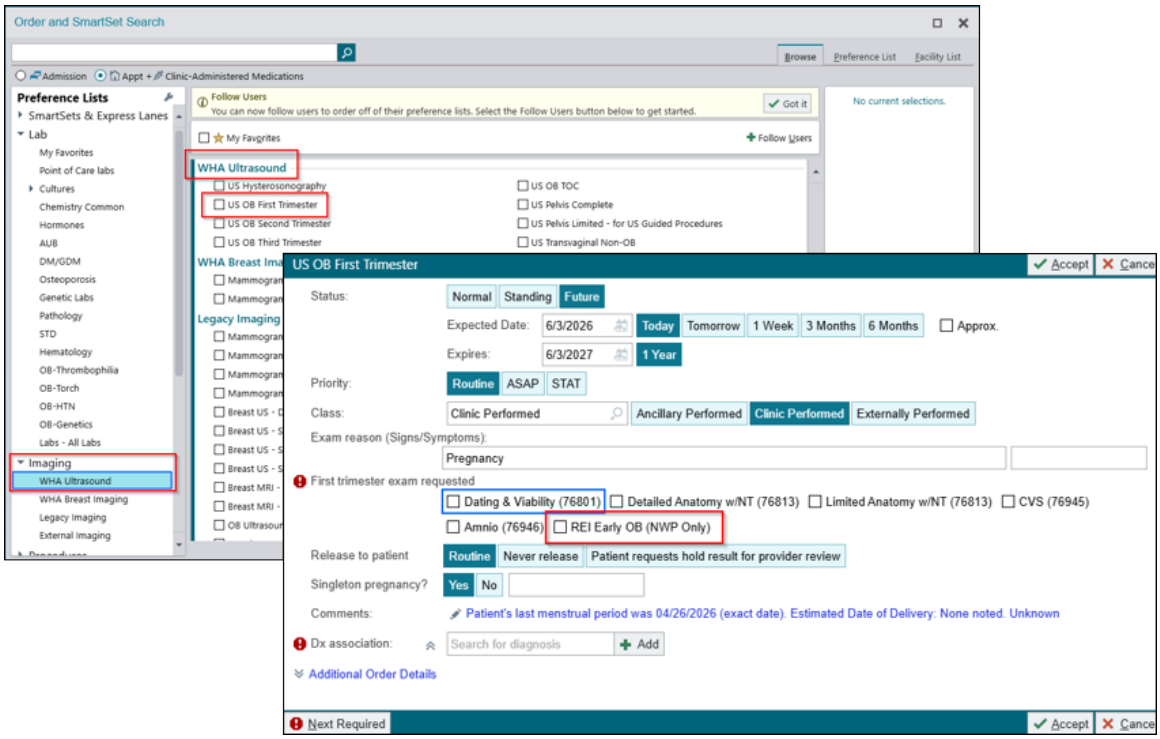
    - Free type in summary to include scheduling details, ultrasound and lab order requests.
9. USC to route to WHA Care Coordinator NWP Pool to review referral, place orders, send scheduling instructions/any additional requests/needs.
10. The CCC updates Care Coordination note in the Snapshot using the SmartPhrase .NWPCCNNW:
    - SmartPhrase includes the following information: PNWF Fertility patient, PNW Fertility provider, OB provider (which will be listed in the referral when received)

- If the patient is not established with an OBGYN provider, the 'OB Provider' does not need to be included in the care coordination note.



11. CCC places ultrasound orders via ancillary orders under the PNWF ordering provider and any lab orders under Dr. Ashlie Tronnes. Screenshots for Ultrasounds orders below.

<u>OB Ultrasound Order:</u>



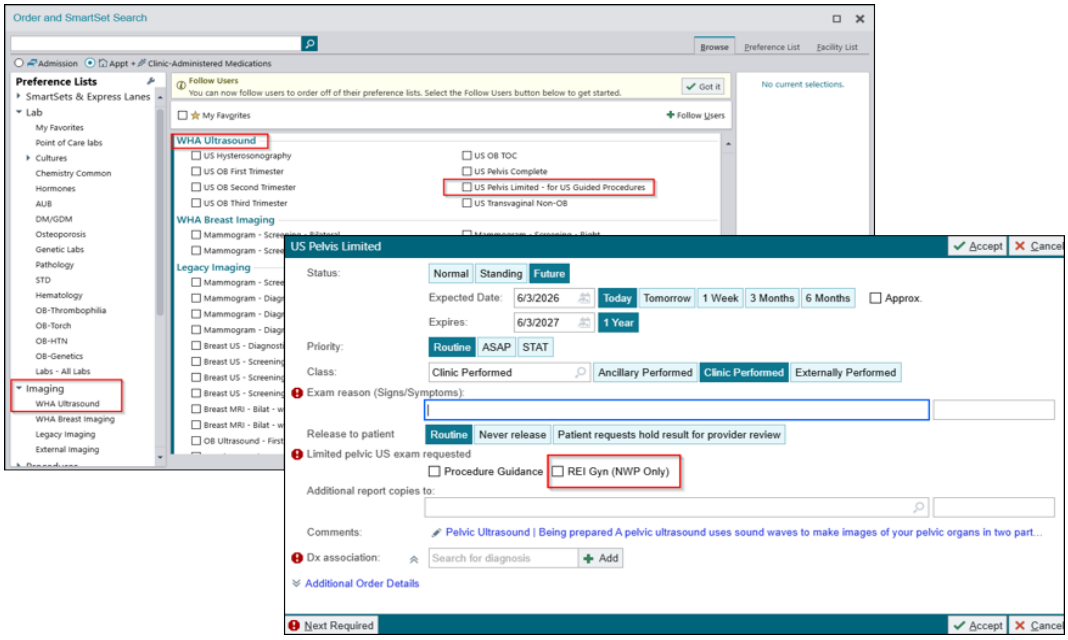
GYN Ultrasound Order:

All labs will be scheduled on NWPW lab or NWPE/TA lab schedules.

If **patient does not have an OB provider listed** this will prompt the USC's to use the following script after getting the patient scheduled:

- *"I see that you don't currently have an OB provider listed. I wanted to let you know that we are accepting new patients. We would be happy to send you a MyHealth message that includes our locations, phone numbers, and the link to our website, so you can schedule at your convenience."*

USC's to document in the NWP patient summary that this was discussed and information via MyHealth was sent to the patient.



CCC routes back to USC pool [WHA US COORDINATORS POOL] and USC calls the patient to schedule (visit types noted below).

NWPW and NWPE US have schedule blocks specific for these PNWF ultrasounds:

West 1: Monday 7:20am

West 1: Wednesday 9:10am

East 1: Friday 7:20am

Visit types:

REI GYN US WHA/NWP – 60 mins

- Needs to be schedulable with a Limited Pelvic US order

REI OB US WHA/NWP – 50 mins

- Needs to be schedulable with a First Trimester OB US order

REI LAB WHA/NWP – same default length as our other lab visit types

Any future requests/orders will be faxed as noted above and the USC's can document updated needs in the original patient summary encounter.

<u>Lab Order Workflow:</u>

- Enter orders when they come in from PNW fertility under Ashlie Tronnes as ordering provider
- Results will come back to NWP lab pool
- RN reviewing incoming lab results will review Care Coordination Note to determine care plan.

- If Care Coordination note indicates PNWF patient, RN will electronically fax results to the PNWF provider via Epic and will done result.

IN-HOUSE tests WHA can Draw/ Collect AND Perform testing in CL:

- FSH
- Estradiol
- Beta HCG Quant (routine and STAT)
- RPR
- TSH
- ABO Blood Group and Rh Type
- Prolactin
- Progesterone
- Free T4
- Hepatitis B Surface Antigen
- Hepatitis C Virus Antibody
- HIV 1/2
- Rubella IgG
- CT/GC- vaginal only

SEND OUT TESTS WHA can Draw/Collect and send to Quest reference lab for testing:

- AMH
- LH
- Hepatitis B Core Antibody Total
- Varicella Zoster Antibodies, IgG
- CMV IgG and IgM Antibodies
- CT/GC- urine and cervical
- TPO (Thyroid Peroxidase) Antibodies

Test WHA cannot collect or test due to limited specimen stability that the patient MUST go to Quest or LabCorp Patient Service Center for collection of:

- Semen Analysis

NWP Scheduling Appointment Length Guide

MFM Consult

- 15min nurse
- 50min w/MFM

OB US w/MFM Consult

- Correct length of US **See US order and US Scheduling Guide
- 15min nurse
- 50min w/MFM

MFM Telehealth NWP

- 15min nurse
- Time w/MFM varies based on type of visit

Genetic Consult w/MFM Consult

- 60min w/GC
- 15min nurse
- 50min w/MFM

Peds Cardio w/MFM Consult

- 60min US
- 25min PEDS Cardiologist
- 15min nurse
- 50min w/MFM

OB Check

- 15min nurse
- 25min w/MFM

OB Check w/NST

- 40min NST
- 25min w/MFM

OB US w/OB Check

- Correct length of US **See US order and US Scheduling Guide
- 15min nurse
- 25min w/MFM

Postpartum NWP/PP Telehealth

- 15min nurse
- 25min w/MFM

Postpartum Triage

- 15min nurse
- 25min w/MFM or 30min w/MOD

Peds Cardio w/OB Check

- 60min US
- 25min PEDS Cardiologist
- 15min nurse
- 25min w/MFM

MOD Risk Consult

- 15min nurse
- 45min w/MOD Provider

MOD Risk Telehealth

- 15min nurse
- 30min w/MOD Provider

MOD Risk Postpartum

- 15min nurse
- 30min w/MOD Provider

MOD Risk Postpartum Telehealth

- 15min nurse
- 30min w/MOD Provider

OB US w/MOD Risk Consult

- Correct length of US **See US order and US Scheduling Guide
- 15min nurse
- 45min w/MOD Provider

MOD Risk Follow up

- 15min nurse
- 30min w/MOD Provider

MOD Risk Follow up w/NST

- 40min NST
- 30min w/MOD

OB US w/MOD Risk Follow up

- Correct length of US **See US order and US Scheduling Guide
- 15min nurse
- 30min w/MOD Provider

NST NWP

- 40min nurse

Nurse Visit

- Varies based on visit type
- Injection/Blood draw=15min nurse
- **Teachings=30min nurse(unless otherwise noted)**
- Nurse Visit w/in-person or phone interpreter =60 mins

Genetic Telehealth NWP

- 60min w/GC
- 10min w/MFM (US reader)
  *MFM and GC must overlap last 10min

Genetic Consult NWP

- 60min w/GC
- 10min w/MFM (US reader)

Genetic Telehealth NWP (sign CVS or Amnio Consents)

30mis w/GC

Genetic Consult w/MFM Consult

- 60min w/GC
- 15min nurse
- 50min w/MFM
  *If TH, nurse and MFM can overlap

Genetic Consult Only NWP

- 60min w/GC

Genetic Testing/Lab Visits -

Amnio only NWP

- 30min consent visit w/GC (no MFM FYI)
- 50min US

Amniocentesis NWP

- 30min consent visit w/GC (no MFM FYI)
- Correct length of US **See US order and US Scheduling Guide

- 20min w/MFM (US reader)
  *MFM overlaps last 20 min of US

CVS NWP

- 30min consent visit w/GC (no MFM FYI)
- Correct length of US **See US order and US Scheduling Guide
- 40min w/Nurse & MFM (US reader)
  *Nurse/MFM overlaps last 40 min of US
- Lab draw

Genetic Consult w/Lab NWP

- 60min w/GC
- 10min w/MFM (US reader)
- 10min Lab

Ultrasound Only Visits

OB Ultrasound NWP

- Correct length of US **See US order and US Scheduling Guide

Satellite Ultrasound NWP

**Only for Satellite Ultrasound scheduling

- 20min w/MFM (US reader)
  *MFM overlaps last 20 min of US
- If noted in scheduling instructions:
- OB Check to follow: 15 min nurse/25 min MFM (US Reader)

Genetic Consult w/OB US FYI NWP

- 60min w/GC
- 10min w/MFM (US reader)
- Correct length of US **See US order and US Scheduling Guide

Lab NWP

- 10 mins

OB Ultrasound

- Correct length of US **See US order and US Scheduling Guide
- 10min w/MFM (US reader)
  *MFM overlaps last 10 min of US

NWP Visit Types/Appointment Notes

Note: Appointment notes in Epic are viewable on Myhealth and the appointment confirmation page. Spell out all words and use limited abbreviations.

For all visits scheduled at NWP **East or** NWP **GW**, appt notes must contain ****NWP Pt**** before all visit notes

Standard Appointment Notes for NWP

- Gestational Age, Type(s) of appointment, reason for appointment & referring provider full name

  Example: 20.1wks Detailed Anatomy – AMA, Lisa Thum

Standard VIDEO VISIT Appointment Notes for NWP

- VIDEO VISIT: Patient aware of text link, Gestational Age, Type(s) of appointment, reason for appointment & referring provider full name, Patient Phone Number

Use more detailed Appointment Notes within the NWP Patient Summary

- Appt Date, US (time), RN (time), provider initials (time) Gestational Age, Type(s) of appointment, reason for appointment & referring provider full name

For examples on how long each visit type should be, see the NWP Scheduling Appointment Length Guide

MFM Visit Appt Notes

MFM Consult NWP – Ex: GA, MFM Consult, Reason, Referring Provider

MFM Telehealth NWP – Ex: VIDEO VISIT: Patient aware of text link. GA, OB Check

PH: 503-297-3660**This visit type is used for MFM OB Check Telehealth, MFM Consult Telehealth

OB Check NWP – Ex: GA, OB Check

OB Check w/NST NWP – Ex: GA, NST and OB Check

Postpartum NWP (for 3wk visits) – Ex: Postpartum 3wk, delivered XX/XX/XXXX

Postpartum Triage NWP (for all other MFM visits other than 3wk) Ex: Postpartum 1 or 2wk, delivered XX/XX/XXXX

PP Telehealth Visit NWP: – Ex: VIDEO VISIT: Patient aware of text link. Postpartum 3wk, delivered XX/XX/XXXX PH: 503-297-3660

Genetic Consult w/MFM Consult NWP – Ex: GA, Genetic Consult and MFM Consult, Reason, Referring Provider

OB Ultrasound w/MFM Consult NWP – Ex: GA, Detailed Anatomy and MFM Consult, Reason, Referring Provider
**Use US type stated on US order in appt notes, Detailed Anatomy is only an example

OB Ultrasound w/OB Check NWP – Ex: GA, Detailed Anatomy and OB Check
**Use US type stated on US order in appt notes, Detailed Anatomy is only an example

Peds Cardio w/MFM Consult NWP – Ex: GA, Fetal Echo, PEDS Cardio and MFM Consult

Peds Cardio w/OB Check NWP – Ex: GA, Fetal Echo, PEDS Cardio and OB Check

MOD Visits

Mod Risk Consult NWP – Ex: GA, MOD Consult, Reason, Referring Provider

Mod Risk Follow up NWP – Ex: GA, MOD Follow Up

MOD Risk Follow up w/NST NWP – Ex: GA, NST and MOD Follow Up

Mod Risk Postpartum NWP– Ex: MOD Postpartum **3wk, delivered XX/XX/XXXX

**PP + # of weeks post delivery date

MOD Risk Postpartum Telehealth NWP- VIDEO Visit: Patient aware of text link. MOD Postpartum **3 wk PH: 503-297-3660
**PP + # of weeks post delivery date

MOD Risk Telehealth NWP – Ex: VIDEO VISIT: Patient aware of text link. MOD Postpartum **3 wk PH: 503-297-3660
**PP + # of weeks post delivery date (Post Partum 3 wk)

OB Ultrasound w/Mod Risk Consult NWP – Ex: GA, Detailed Anatomy and MOD Consult, Reason, Referring Provider
**Use US type stated on US order in appt notes, Detailed Anatomy is only an example

OB Ultrasound w/Mod Risk Follow up NWP – Ex: GA, Detailed Anatomy and MOD Follow Up
**Use US type stated on US order in appt notes, Detailed Anatomy is only an example

Nurse Only Visits

NST NWP – Ex: GA, NST Only, Type of Patient (MFM or MOD)

Nurse Visit NWP – Ex: GA**,** Nurse Visit – Type of Visit
**Injection, Insulin Teaching = 30min (*if with Interpreter 45min, or length of time requested by Nurse), FHT Checks, etc

Genetic Consult Visits

Genetics Telehealth NWP – Ex: VIDEO VISIT: Patient aware of text link. GA, Genetic Consult, Reason for consultation, Referring Provider PH: 503-297-3660

**also used for Genetic Consult for consents (CVS, Amnio) 30min and no MFM needed

Genetic Consult NWP – Ex: GA, Genetic Consult, Reason for consult, Referring Provider
**In-person consult

Genetic Consult Only NWP – Ex: GA, Genetic Consult Only, Reason, referring provider
**In-person consult with no US Reader

Genetic Consult w/MFM Consult NWP– Ex: GA, Genetic Consult and MFM Consult, Reason for consults, Referring Provider

Genetic Testing/Lab Visits

Amnio only NWP – Ex: GA, Amnio Only w/ MFM Name
**Only used for repeat or Amnio only, consist of Ultrasound and US Reader. *GC visit needed before Amnio to sign consents if not already completed*, appt notes for GC visit Ex: Amnio Consents

Amniocentesis NWP – Ex: GA, Amnio w/MFM Name
**GC visit needed before Amnio to sign consents, appt notes for GC visit Ex: Amnio Consents

CVS NWP – Ex: GA, CVS w/MFM Name
**GC visit needed before CVS to sign consents, appt notes for GC visit Ex: CVS Consents

Genetic Consult w/OB US FYI NWP: *Appt visit type rarely used. If needed, use NWP Standard Appointment Notes*

OB Ultrasound w/Lab NWP *– Ex: Sequential 1 Ultrasound and Lab draw (not used as often)*

Lab NWP: Leave blank – no appointment notes with lab draw type! (Unless at NWP East or NWP Gateway – add **NWP PT**)

Ultrasound Only Visits

OB Ultrasound NWP (50, 60, 70, 110, 120, 130, 140, 160) – Ex: GA, TVCL Only
**Use US type stated on US order in appt notes, TVCL is only an example

OB Ultrasound (50, 60, 70, 110, 120, 130, 140, 160) w/FYI NWP – Ex: GA, Fetal Echo and FYI to MFM Name
**Use US type stated on US order in appt notes, Fetal Echo is only an example

Satellite Ultrasound NWP – Ex: TOC
**Only for Satellite Ultrasound scheduling